



ILLNESS, INJURY & MEDICAL EMERGENCY POLICY MANUAL

Wellness Monitoring · Illness and Injury Response · Medical Emergency Procedures · Hospital Transfer · Serious Illness and Injury Notification — MCFD Social Worker and Licensing (M) / CLBC Critical Incident Reporting (C)

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Effective Date	July 1, 2026
Review Date	July 1, 2027 (annual); also upon any funder or licensing directive, any serious illness/injury or medical emergency incident, or BC legislative or regulatory change
Applies To	All Evergreen Community Care residential care programs and 1–2 bed homes serving: children and youth in MCFD care; adults receiving CLBC-funded residential services. All Youth Care Workers / Community Support Workers, Shift Leads, Program Coordinators, Program Managers, and the Executive Director.
Funder Streams	M = MCFD (children and youth) · C = CLBC (adults) · Both = both streams. Where a requirement is stream-specific it is marked (M) or (C).
Legislative Authority	Community Care and Assisted Living Act and Residential Care Regulation (reportable incidents — where a facility is licensed, 3+ persons in care); Child, Family and Community Service Act (CFCSA) s.14 reporting duty (M); Adult Guardianship Act (C); Health Care (Consent) and Care Facility (Admission) Act — emergency provisions; Infants Act framework (M); Occupational Health and Safety Regulation (WorkSafeBC first aid requirements); Coroners Act (BC)
Funder Standards	MCFD Standards for Staffed Children's Residential Services (SHSS): Standard D (health and wellbeing); reportable circumstances and

	critical injury/illness reporting requirements. CLBC contract terms: critical incident reporting, health and safety provisions per the executed service contract.
Related Documents	Medication Administration Manual (ECC-MED-2026-001) — anaphylaxis/EpiPen §23, Naloxone §23.3, adverse drug reactions §36, medication emergencies §37, hospital transfer §34; Health Care & Consent Policy Manual (ECC-HC-2026-001) — emergency care without consent (Part 5), consent frameworks, prescriber attachment; Nutrition, Illness & Infection Prevention Manual (ECC-NIP-2026-001) — illness exclusion and outbreak; Behaviour Support Manual (ECC-BSP-2026-001) — crisis de-escalation; OHS Program Binder (ECC-OHS-POL-2026-002); 1-2 Bed Home Opening & Compliance Binder (ECC-12BED-2026-001)

 This manual governs how staff recognize, respond to, and report illness, injury, and medical emergencies for every person in care. Three rules govern everything in it: **(1) Call 911 first for any emergency — consent, notification, and documentation always come after. (2) Escalate early — 811, the prescriber, urgent care, and 911 are always available, and no staff member will ever be criticized for escalating in good faith. (3) A serious illness or injury is not fully handled until the required notifications are made: the MCFD social worker and — where the facility is licensed — the Licensing Officer (M), or CLBC per the critical incident terms of the contract (C).** The Section 17 notification matrix is a hard requirement, and the PM personally confirms completion for every event.

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VERSION HISTORY

PART 1 — Policy Foundation

1. POLICY STATEMENT AND GOVERNING PHILOSOPHY

People in residential care depend on the adults around them to notice when something is wrong and to act on it without delay. Illness and injury response is therefore a core competency of every staff member — not a specialist function. This manual sets one agency-wide standard for recognition, response, escalation, hospital care, and notification, so that the response to a health event never depends on which staff member happens to be on shift.

Principle	What It Means in Practice
911 first, everything else after	For any emergency, the call to 911 precedes consent processes (ECC-HC Part 5), supervisor permission, notification, and documentation. No approval is ever required to call 911.
Escalate early and without penalty	The escalation ladder — observe and document → 811 / prescriber → urgent care → 911 — is always open. Over-escalation in good faith is never criticized; under-escalation is a training matter the PM addresses supportively.
Every symptom report is acted on	A person's report of pain or illness is believed and responded to. Symptoms are never screened for credibility, attributed to behaviour, or 'watched' without documentation and a plan.
Know the person's baseline	Recognition depends on knowing what is normal for each person — communication style, activity, appetite, and how they show pain. Staff learn the baseline from the Health Care Profile (ECC-HC App. A) and the Emergency Information Sheet (Appendix D).
Notification is part of the response	A serious illness or injury is closed only when: the person is safe and cared for; the PC and PM know; the MCFD social worker and licensing (where licensed) (M) or CLBC (C) have been notified within the required timelines; and the written report is complete.
Protection duties override procedure	Any injury or illness pattern raising a protection concern engages CFCSA s.14 (M) or Adult Guardianship Act responses (C) immediately. No internal process ever delays a protection report.
Dignity in illness	Ill and injured people receive privacy, comfort, reassurance, and age-appropriate honest information. Care is provided with the person, not merely to them.
Learn from every event	Every reportable event is reviewed by the PM; every review can generate corrective action. The purpose is system improvement, never blame — mirroring ECC-MED §31.

2. LEGISLATIVE AND REGULATORY FRAMEWORK

Authority	Stream	Relevance and Key Requirements
MCFD SHSS — reportable circumstances / critical injury and illness reporting	M	Prescribed incidents — including serious injury, serious illness requiring medical attention or hospitalization, medical emergencies, and any incident affecting a child's safety — are reported to the MCFD social worker (after hours: Centralized Screening 1-800-663-9122) within required timelines, with the written report to follow. The PM maintains the current MCFD reportable circumstances list at the program.
CLBC service contract — critical incident reporting	C	Critical incidents — including serious injury, unexpected serious illness, medical emergencies, hospitalization, and death — are reported to CLBC per the executed contract: prompt verbal notification and a written critical incident report within the contract timeline (commonly 24 hours). The PM extracts the contract's exact CI definitions and timelines and posts them with Appendix F.
Residential Care Regulation — reportable incidents	Both (licensed facilities only)	Licensed facilities report prescribed incidents (including serious illness and injury) to the Licensing Officer. A 1-2 bed home is not licensed; this manual adopts the practice standard everywhere, and the licensing column of the notification matrix applies only where a licence exists.
CFCSA s.14	M	Anyone with reason to believe a child needs protection must report to a child welfare worker. Applies to injuries of unknown origin, injuries inconsistent with explanation, and patterns of injury or untreated illness.
Adult Guardianship Act	C	Abuse, neglect, or self-neglect of an adult who cannot seek support is reported to the designated agency

Authority	Stream	Relevance and Key Requirements
		(health authority; CLBC in some contexts). Applies to the same injury/illness patterns in the adult stream.
HCCCFAA s.12 / Infants Act framework	Both	Health care providers may provide emergency care without consent to preserve life or prevent serious harm (ECC-HC Part 5). Program practice: 911 first; consent contacts notified after.
WorkSafeBC OHS Regulation	Both	Workplace first aid requirements (attendants, kits, records) per the OHS Program Binder; staff injuries follow OHS procedures in parallel with this manual.
Coroners Act	Both	Certain deaths must be reported to the BC Coroners Service; the scene and medications are secured pending direction (Section 18; ECC-MED §34).

3. ROLES AND RESPONSIBILITIES

Role	Responsibilities Under This Manual
Youth Care Worker / Community Support Worker	Complete wellness monitoring every shift and document; respond to illness and injury per Parts 4–6 within first aid training; call 911 for any emergency without seeking permission; notify the PC/on-call the same shift for anything beyond minor first aid; complete the Illness/Injury Report (Appendix A) before end of shift; know each person's Emergency Information Sheet (Appendix D) and where it is kept; maintain current first aid certification.
Shift Lead	All worker responsibilities plus: lead the on-scene response to any medical event; assign roles (caller, first aider, supervision of other residents); initiate the notification chain (Section 17) immediately after the person is safe; ensure the Emergency Information Sheet travels with any transfer; complete an explicit verbal handover of any ongoing observation (e.g., head injury checks) to the incoming shift.
Program Coordinator (PC)	Receive all illness/injury notifications and direct next steps; complete PC-level notifications and support the PM's funder notifications; maintain Appendix D sheets current for every person and the posted Appendix C/F references; track follow-up care to completion (with

Role	Responsibilities Under This Manual
	ECC-HC App. E); audit Appendix A reports monthly for completeness; arrange debriefs for staff after distressing events.
Program Manager (PM)	Make or personally confirm every funder notification in the Section 17 matrix; review every Appendix A report within 24 hours and every emergency or hospitalization within 5 business days; maintain the current MCFD reportable circumstances list (M) and the CLBC contract CI terms (C) posted at the program; ensure first aid coverage and training currency; deliver corrective actions with named owners; report trends quarterly to the ED.
Executive Director (ED)	Approve this manual annually; receive quarterly illness/injury/emergency trend reports; ensure resourcing (staffing for hospital accompaniment, first aid training budget, equipment); respond to licensing or funder directives; support the PM on any death, protection matter, or media-sensitive event.

4. KEY DEFINITIONS

Term	Definition as Used in This Manual
Minor illness / minor injury	An illness manageable with home comfort care, or an injury fully managed by program-level first aid, with no red flags and no clinical involvement required beyond optional 811 advice.
Serious illness	An illness requiring emergency care, hospital assessment or admission, or identified as serious by a prescriber; or any illness with rapid deterioration, altered consciousness, breathing difficulty, uncontrolled pain, or dehydration risk. Always reportable per Section 17.
Serious injury	An injury requiring medical treatment beyond program-level first aid — suspected fractures, head injury with any red flag, burns beyond minor first-degree, wounds requiring closure, dental trauma, or any injury requiring hospital assessment. Always reportable per Section 17.
Medical emergency	Any condition in the Section 14 table — an immediate threat to life or of serious harm requiring 911.
Reportable circumstance (M)	Any event on the current MCFD reportable circumstances list. The list, maintained by the PM, governs; this manual's matrix is the program's operationalization of it.
Critical incident (C)	Any event meeting the CLBC contract's critical incident

Term	Definition as Used in This Manual
	definitions. The contract governs; the PM posts its definitions and timelines with Appendix F.
Injury of unknown origin	Any injury without a witnessed or credibly explained mechanism, or where the explanation is inconsistent with the injury. Triggers Section 13 in full.
Escalation ladder	The standing sequence of clinical options: observe and document → 811 (HealthLink BC) or prescriber → urgent care / UPCC / emergency department → 911.

PART 2 — Prevention and Preparedness

5. FIRST AID AND EMERGENCY READINESS

Requirement	Standard
Staff certification	Every staff member holds a current first aid certificate appropriate to the ages served before any unsupervised shift (ECC-12BED §2.4); the roster is built so someone with current certification is on site at all times; currency is tracked in the monthly staff currency check (by the 5th).
First aid kits	A stocked kit at the home and in every program vehicle; contents per the OHS Program Binder standard; checked monthly on the facility checklist; used items replaced immediately; kit locations known to all staff and shown in orientation.
Emergency equipment	Where prescribed for a person in care: EpiPen stored unlocked, labelled, accessible (ECC-MED §23.2); Naloxone kit where opioids are in use (ECC-MED §23.3); rescue medications per individual plans; program thermometer, and any person-specific equipment (glucometer, blood pressure cuff) per clinical direction — all checked monthly.
Posted information	Beside every phone: 911, Poison Control 1-800-567-8911, 811, on-call supervisor schedule, prescriber after-hours, MCFD Centralized Screening 1-800-663-9122 (M), CLBC contact (C), and the Appendix C quick reference. The Appendix F notification matrix poster is displayed in the staff area.
Vehicle and route knowledge	Staff know the route to the nearest emergency department and UPCC; the vehicle is fuelled and roadworthy (monthly check per ECC-12BED §4); car seats/restraints per MCFD requirements (M).

6. EMERGENCY INFORMATION SHEETS

Every person in care has a one-page Emergency Information Sheet (Appendix D) prepared by the PC, stored where all staff can retrieve it in seconds, and updated within 1 business day of any change. It is the single document that travels with the person in any emergency — to hospital, with paramedics, or on any transfer — and contains: identity and PHN; allergies; diagnoses; current medications (kept synchronized with the MAR per ECC-MED); the person's baseline and communication needs; consent framework and contacts (M: social worker and after-hours line; C: substitute decision-maker contacts per ECC-HC Appendix C); prescriber and pharmacy. The Shift Lead verifies at handover that the sheets are in their known location.

7. DRILLS, MOCK SCENARIOS AND THE ON-CALL SYSTEM

- A medical emergency tabletop or drill is run at least annually at every program (and before opening per ECC-12BED §2.5): a realistic scenario (anaphylaxis at dinner; seizure; unresponsive person) walked through to the end of the notification chain and documentation. Findings generate corrective actions.
- New staff walk through the emergency response — kit locations, Appendix D sheets, posted numbers, their role in a two-staff and single-staff response — during orientation before unsupervised duties.
- An on-call supervisor (PC or PM) is reachable 24/7; the weekly on-call schedule is posted; the on-call number is tested monthly. Single-staffed shifts have a documented plan for supervising other residents during an emergency (a neighbouring staffed resource, on-call attendance, or EMS direction).
- After any significant emergency, the PC offers staff (and supports residents with) a debrief; critical incident stress support is arranged through the PM where needed (HR Policy Binder).

PART 3 — Wellness Monitoring and Early Recognition

8. SHIFT WELLNESS MONITORING

- Staff observe each person's wellness every shift — appetite and fluids, sleep, energy, mood, pain, skin, breathing, elimination where relevant — and document meaningful observations in the shift log using the program's daily log / wellness-check format (ECC-12BED §2.5). 'Fine all shift' is acceptable only when it is true and based on actual contact.
- Wellness observations are also medication-monitoring data: appetite, sedation, movement, and mood changes are cross-read against ECC-MED §24 and §35 requirements for each person's medications.
- Any concerning change is reported to the PC the same shift and handed over explicitly and verbally at shift change — an entry in the log alone is not a handover.
- Persons in care are supported to understand and report their own symptoms; being believed is program policy. Staff respond to every symptom report with assessment and documentation, never with dismissal.

9. RECOGNIZING CHANGE IN PEOPLE WHO DO NOT SELF-REPORT

Practice	Standard
Know the baseline	For each person who does not reliably report symptoms verbally (young children (M); adults with communication differences (C)), the Health Care Profile and Appendix D record their baseline and their known indicators of pain or illness (changes in eating, sleep, activity, vocalization, irritability, guarding, withdrawal, self-injurious behaviour changes).
Behaviour change is a health question first	A significant unexplained behaviour change prompts a

Practice	Standard
	health screen before a behavioural interpretation: pain source check (teeth, ears, abdomen, skin, feet), constipation, urinary symptoms, menstrual pain, medication side effects (ECC-MED §35), infection. The PC arranges prescriber review where the screen does not resolve it. This ordering is deliberate: untreated pain presenting as 'behaviour' is a known and preventable failure in residential care.
Objective measurement	Where symptoms are suspected, staff use objective measures within their training: temperature, fluid intake/output observation, visible signs — documented with times. Program-initiated vital-sign regimes beyond this occur only on clinical direction.
Low threshold for advice	For a person who cannot describe their symptoms, the threshold for calling 811 or the prescriber is deliberately lower — the history is weaker, so professional assessment matters more.

PART 4 — Response to Illness

10. THE ESCALATION LADDER

Level	Presentation (examples)	Response
Level 1 — Home comfort care	Common cold, mild sore throat, low-grade fever settling with fluids and rest, mild GI upset, minor rash without other symptoms	Comfort care: rest, fluids, monitoring each shift. Fever/pain medication ONLY per the person's authorized OTC/PRN protocols (ECC-MED §18-19) — never improvised dosing. Apply exclusion criteria (ECC-NIP Appendix G). Document in the shift log. PC informed same shift.
Level 2 — Clinical advice same day	Fever persisting beyond 24–48 hours or 38.5°C+ not settling; persistent vomiting or diarrhea; significant or persistent pain; wheeze without distress; suspected infection (ear, urinary, skin, chest); any notable illness in a person with significant chronic conditions or on high-alert medications	Call 811 (HealthLink BC) or the prescriber's office the same day; follow and document the advice received; arrange same-day or next-day appointment as advised; increase monitoring frequency with documented checks; PC notified same shift; consent per ECC-HC frameworks.
Level 3 — Urgent care / emergency department today	Signs of dehydration (minimal urine, lethargy, dry mouth, sunken eyes); breathing difficulty short of distress; uncontrolled pain; moderate injury per Section 12;	Transport to urgent care/UPCC or ED without delay; staff member accompanies; Appendix D Emergency Information Sheet travels; PC/PM notified per Section

Level	Presentation (examples)	Response
	clinical advice directs assessment	17; other residents' supervision arranged per the Section 7 plan.
Level 4 — 911	Any Section 14 criterion	911 immediately; Part 6 procedures.
 Choosing a higher rung than needed is never criticized. If two staff disagree about severity, the higher assessment governs. If a person's condition is worsening while you deliberate — stop deliberating and go up a level.		

11. SYMPTOM-SPECIFIC GUIDANCE

Symptom	Standard
Fever	Measure with the program thermometer; document temperature, route, and time. Antipyretics only per the authorized OTC/PRN protocol. Escalate to Level 2 for fever persisting 24–48 hours, 38.5°C+ not settling, or fever with significant illness signs. Emergencies: fever with stiff neck, severe headache, non-blanching rash, seizure, or unusual drowsiness → 911; fever in an immunocompromised person → same-day clinical direction, low threshold for ED.
Vomiting / diarrhea and dehydration	Small frequent sips of fluid (oral rehydration solution where available); monitor and document intake and urine output; exclusion per ECC-NIP Appendix G; 2+ linked cases triggers the ECC-NIP outbreak protocol. Dehydration red flags (minimal or no urine over 8 hours, lethargy, sunken eyes, dizziness on standing) → Level 3; unable to keep any fluid down and deteriorating → Level 3–4. Blood in vomit or stool (more than streaks) → same-day clinical advice minimum; significant amounts → ED.
Respiratory symptoms	Mild congestion/cough → Level 1 with monitoring. Wheeze in a person with asthma: rescue inhaler per their plan (ECC-MED §16/§20) and document response; no improvement or repeated need → Level 2–3. Increased work of breathing (fast breathing at rest, indrawing, unable to speak full sentences, blue tinge) → 911.
Pain	Every pain report is assessed (location, severity, onset, what makes it better/worse), documented, and responded to. Analgesia only per authorized protocols. Severe pain, chest pain, testicular pain, sudden severe headache ('worst ever'), or abdominal pain with rigidity, guarding, or fever → Level 3–4. Recurrent or persistent pain → prescriber review, never normalized.
Rash	Rash without other symptoms → Level 1–2 per

Symptom	Standard
	appearance and spread. Any rash in a person on lamotrigine → hold per ECC-MED §24 and call the prescriber immediately. Non-blanching rash (does not fade under pressure), rash with fever and illness, or rapidly spreading hives → Level 3–4; hives with any breathing/swallowing involvement → anaphylaxis protocol (Section 15).
Mental health symptom changes	New or worsening mood, anxiety, psychotic, or self-harm symptoms are health events: documented objectively, reported to the PC same shift, prescriber engaged per urgency (this is also required medication monitoring for antidepressants — ECC-MED §22). Imminent risk → Section 15 crisis protocol.

PART 5 — Response to Injury

12. INJURY RESPONSE BY SEVERITY AND THE HEAD INJURY PROTOCOL

Injury	Response
Minor (small cuts, scrapes, minor bruises, small first-degree burns)	First aid per training; document in the shift log and the minor-injury record with body-map notation (Appendix E) where useful; monitor; PC informed same shift. Repeated minor injuries in one person are a pattern to review, not a series of shrugs.
Requires medical attention (wounds possibly needing closure; suspected sprain or fracture; burns beyond minor; dental trauma; animal or human bites breaking skin; eye injuries; any injury with significant pain, deformity, or loss of function)	First aid within training (immobilize suspected fractures as trained; do not attempt reduction); clinical advice (811/prescriber) or direct transport to urgent care/ED per severity; staff member accompanies with Appendix D; complete Appendix A with body map (Appendix E); notifications per Section 17.
Serious / emergency (major bleeding; suspected spinal injury; compound fracture; serious burns; head injury red flags; unconsciousness)	911; first aid within training — direct pressure for bleeding; do not move a suspected spinal injury except for immediate danger or airway; burns cooled with running water (not ice); Part 6 applies; notifications per Section 17.

12.1 Head Injury Protocol

Step	Standard
1. Assess for red flags	911 / immediate ED for ANY of: loss of consciousness (any duration); repeated vomiting; worsening or severe headache; confusion, unusual drowsiness, or behaviour change; seizure; unequal pupils; weakness, numbness, or unsteadiness; clear fluid or blood from ears or nose; fall from height or other high-mechanism injury; any head injury in a person on anticoagulants

Step	Standard
	(ECC-MED §24) or with a bleeding disorder.
2. No red flags — clinical advice anyway	Call 811 or the prescriber the same day for any head injury beyond a trivial bump, describe the mechanism and the person, and follow the advice given. Document the advice verbatim.
3. Structured observation	Begin the Head Injury Observation Record (Appendix B): checks at the advised intervals (default: hourly for 4 hours, then every 4 hours to 24 hours, including waking checks overnight if directed) covering alertness, orientation/behaviour vs baseline, headache, vomiting, pupils as trained, and movement. Every check is documented with time and initials.
4. Explicit handover	The observation schedule is handed over verbally at every shift change until complete; the incoming staff member signs the Appendix B record to accept it.
5. Any red flag at any point	→ 911. The observation record travels with the person.

13. INJURIES OF UNKNOWN ORIGIN AND PROTECTION DUTIES

Requirement	Standard
Treat first	The injury is treated per Section 12 severity — protection steps run in parallel, never instead of care.
Objective documentation	Document exactly what is observed: location, size, colour, shape (Appendix E body map); the explanation given and by whom, verbatim; the setting in which it was discovered. Photographs only per program practice and the applicable consent framework (ECC-HC §29). No speculation in the record — observations and quotes only.
Immediate escalation	PC and PM are notified immediately (not end of shift) for any injury of unknown origin or inconsistent with its explanation.
Protection reporting (M)	Where there is any reason to believe the child may need protection, the CFCSA s.14 duty applies to the individual staff member who holds the belief — it is reported to a child welfare worker directly and cannot be delegated up the chain or vetoed by a supervisor. The social worker is also notified per Section 17.
Protection reporting (C)	Suspected abuse or neglect of an adult is reported to the Adult Guardianship Act designated agency (health authority) and to CLBC per the contract's critical incident terms; police are called where a crime may have occurred.

Requirement	Standard
Scene and evidence	Where abuse or assault is suspected: preserve clothing/items as practicable, limit discussion with the person to support (no investigative interviewing — that belongs to the authorities), and document who was present and when.
Pattern review	The PM's monthly review of injury records specifically looks for patterns by person, location, time of day, and staffing — a pattern is reportable even where each individual event seemed minor.

PART 6 — Medical Emergencies

14. 911 CRITERIA AND STAFF ACTIONS DURING AN EMERGENCY

Call 911 immediately for	While waiting for EMS
Unresponsiveness or altered consciousness; not rousable	Airway–breathing–circulation per first aid training; CPR if trained and indicated; recovery position if breathing with no spinal concern; never leave the person
Difficulty breathing; choking not relieved by first aid; blue lips or fingertips	Choking first aid per training; position of comfort for breathing difficulty; assist with the person's rescue inhaler if prescribed (ECC-MED \$16/\$20)
Anaphylaxis	EpiPen FIRST — see Section 15
Suspected overdose or poisoning	Poison Control 1-800-567-8911 simultaneously; Section 15
Seizure over 5 minutes, first-ever seizure, repeated seizures, or seizure with injury or breathing difficulty	Section 15 — do not restrain; protect and time
Chest pain; signs of stroke (FAST: Face droop, Arm weakness, Speech difficulty — Time to call 911)	Keep the person still and calm; record symptom onset time for paramedics
Major bleeding; major trauma; serious burns; suspected spinal injury	Direct pressure for bleeding; cool burns with running water; do not move suspected spinal injury unless in immediate danger
Suicide attempt or serious self-harm	Stay with the person; remove immediate dangers as safely possible; calm supportive presence; Section 15

14.1 Staff Actions — The Emergency Sequence

Order	Action
1	Make the scene safe within your ability; do not create a second casualty.
2	Call 911 (or direct a second staff member/capable resident to call while you provide first aid). Speaker phone on; follow dispatcher instructions; do not hang

Order	Action
	up until told.
3	Provide first aid within your training. Send someone to unlock the door, secure pets, and meet EMS.
4	Retrieve the person's Emergency Information Sheet (Appendix D); give it to paramedics; it travels with the person along with current medications per ECC-MED §34 where directed.
5	Arrange supervision of other residents per the Section 7 plan; reassure them honestly and age-appropriately.
6	A staff member accompanies the person to hospital wherever staffing allows and stays until hand-off is confirmed (ECC-HC Part 5).
7	Notify: PC/on-call within 30 minutes → PM → Section 17 matrix. Consent contacts are notified the same shift so the hospital can engage them for decisions beyond the emergency.
8	Document on Appendix A before end of shift: times, observations, actions, EMS details, notifications.
 911 is never delayed for consent, notification, documentation, or supervisor permission. Call first. Notify after (ECC-HC Part 5). No staff member will ever face criticism for calling 911 in good faith.	

15. CONDITION-SPECIFIC EMERGENCY PROTOCOLS

Emergency	Protocol
Anaphylaxis	EpiPen immediately per ECC-MED §23.1 — orange tip to outer thigh, hold 10 seconds — before or simultaneous with calling 911. Never wait to see if symptoms improve. Second dose in the opposite thigh after 5–15 minutes if no improvement and available. Lying down with legs elevated unless breathing is easier sitting; do not let the person stand or walk. All anaphylaxis goes to hospital even if apparently recovered — biphasic reactions occur.
Choking	Encourage coughing if effective. Ineffective cough: back blows/abdominal thrusts per current first aid training for the person's age/size; 911 if not immediately relieved or if the person loses consciousness (then CPR per training). Every choking event requiring intervention is documented (Appendix A) and triggers review of the person's diet/texture plan (ECC-NIP Appendix B).
Seizure	Time it from the start. Do not restrain; do not put anything in the mouth; clear hazards; cushion the

Emergency	Protocol
	head; recovery position when movements stop. Follow the person's individual seizure plan including rescue medication only if prescribed and the staff member is authorized (ECC-MED). 911: over 5 minutes, first-ever seizure, repeated seizures without recovery, seizure in water, injury, or breathing difficulty after. Every seizure is documented (time, duration, description, recovery) and reported to the PC and, per plan, the prescriber (ECC-MED §24).
Diabetic emergency	Hypoglycemia (shaky, sweaty, pale, confused, drowsy): follow the person's documented protocol — fast-acting sugar if fully conscious and able to swallow; recheck per protocol. Unresponsive, seizing, or unable to swallow: 911 — nothing by mouth. Hyperglycemia red flags (vomiting, deep rapid breathing, fruity breath, drowsiness): urgent clinical contact / 911. All readings and treatments documented per the protocol (ECC-MED §24).
Suspected overdose / poisoning	911 and Poison Control 1-800-567-8911 (simultaneously or immediately after). Do not induce vomiting unless Poison Control directs. If opioids suspected: Naloxone per training and ECC-MED §23.3; recovery position; rescue breathing if not breathing; Naloxone wears off — hospital assessment is mandatory regardless of recovery. Containers travel to hospital. Medication-related events also engage ECC-MED §29–30 (error response and reporting).
Sepsis awareness	Infection plus any of: very high or very low temperature, rapid breathing, rapid heart rate, confusion or unusual drowsiness, mottled/ashen/blue-tinged skin, minimal urine, or 'the worst they have ever looked' → treat as an emergency, and say the word 'sepsis' to the 911 dispatcher and ED staff.
Cardiac / stroke	Chest pain, pressure, or pain radiating to arm/jaw/back with sweating or nausea → 911; keep still; do not drive the person yourself. Stroke: FAST screen; note last-known-well time; 911 — stroke treatment is time-critical.
Mental health crisis — imminent risk	911 where there is imminent risk to the person or others (request mental-health-trained responders where available). A staff member stays with the person, de-escalates per ECC-BSP, removes means of harm as safely possible, and follows the person's crisis/safety plan. Involuntary processes under the Mental Health Act are physician- and hospital-led (ECC-HC §29); certification is reportable to the funder. Suicide attempts and serious self-harm are reportable

Emergency	Protocol
	per Section 17 and engage the program's critical incident and debrief procedures.

PART 7 — Hospital, Notification and Reporting

16. HOSPITAL TRANSFER AND ACCOMPANIMENT

- A staff member accompanies the person to hospital wherever staffing allows, remains until hand-off to hospital staff (and consent contacts where they attend) is confirmed, and keeps the PC informed. Where the program cannot send an escort immediately, the PC/on-call arranges attendance at the hospital as soon as possible.
- The Appendix D Emergency Information Sheet, current medication list, and allergy record travel with the person (ECC-MED §34; ECC-HC §10). Medications are sent per ECC-MED §34.
- Consent for treatment at hospital follows ECC-HC: emergency care proceeds without consent (Part 5); decisions beyond the emergency revert to the correct consent authority — the program's job is accurate contacts, not consent.
- Notification per Section 17 begins the same shift; the MCFD social worker (M) or the adult's substitute/family contacts (C) are engaged so the hospital can reach the decision-maker.
- On return: discharge instructions are obtained in writing; medication changes are reconciled per ECC-MED §34 before the prior regimen resumes; follow-up appointments enter the ECC-HC Appendix E log; the PM's 5-business-day review (Section 20) is completed jointly with the ECC-HC §22 review.

17. NOTIFICATION REQUIREMENTS — SERIOUS ILLNESS AND INJURY

Notification is part of the emergency response, not an administrative afterthought. The Shift Lead starts the chain the moment the person is safe; the PC and PM complete it; every notification is documented on Appendix A with names and times. The matrix below is posted at every program as Appendix F.

Event	Internal	MCFD stream (M)	CLBC stream (C)
Serious illness (emergency care, ED assessment, or admission)	PC immediately; PC → PM within 30 minutes	MCFD social worker same day (after hours: Centralized Screening 1-800-663-9122); written reportable circumstance report per MCFD requirements (within 24 hours); Licensing Officer notified per RCR where the facility is licensed	CLBC verbal notification promptly (same day); written critical incident report per contract (commonly within 24 hours); substitute decision-maker/family contacts per ECC-HC
Serious injury (treatment beyond first aid)	Same as above	Same as above; injuries of unknown origin additionally engage CFCSA s.14 (Section 13)	Same as above; suspected abuse/neglect additionally engages the Adult Guardianship Act designated agency (Section 13)
Any 911 call / medical	PC immediately; PM	Social worker same day;	CLBC per contract CI

Event	Internal	MCFD stream (M)	CLBC stream (C)
emergency	within 30 minutes	written report within 24 hours; licensing where licensed	terms
Hospitalization (any admission)	PC and PM same shift	Social worker same day; ECC-MED §34 and ECC-HC §10 procedures	CLBC per contract; ECC-HC Part 4/5 contacts
Suicide attempt / serious self-harm	PC and PM immediately	Social worker same day (immediately where safety planning is needed); written report within 24 hours	CLBC immediately per contract; clinical follow-up arranged
Choking requiring intervention; anaphylaxis; overdose; seizure requiring 911	PC immediately; PM within 30 minutes	Reportable same day; medication-related events also per ECC-MED §29–30	Critical incident per contract; medication-related events also per ECC-MED §29–30
Death of a person in care	PM and ED immediately	Section 18 in full	Section 18 in full
 The PM personally confirms that every notification required by this matrix was made and documented — 'I assumed the PC called' is a finding, not an explanation. Where the MCFD reportable circumstances list or the executed CLBC contract is stricter or more specific than this table, the funder document governs; the PM keeps both posted with Appendix F and reconciles this manual to them at each annual review.			

18. DEATH OF A PERSON IN CARE

Step	Action
1	Call 911 unless death has been formally expected and a clinical protocol directs otherwise (e.g., palliative plan with MOST documentation — ECC-HC §29). Follow dispatcher direction, including resuscitation instructions.
2	Do not disturb the scene beyond what care requires. Secure the area; secure all medications immediately and dispose of nothing (ECC-MED §34); preserve documents and devices as found.
3	Notify the PM immediately; the PM notifies the ED immediately.
4	(M) Notify MCFD immediately (social worker / after-hours 1-800-663-9122); notify the Licensing Officer immediately where the facility is licensed. (C) Notify CLBC immediately per the contract; notify the committee/representative and family contacts per ECC-HC — coordinated with police/coroner direction on timing.

Step	Action
5	The BC Coroners Service is engaged per the Coroners Act (police/EMS typically initiate); the program cooperates fully and follows all direction.
6	Support the other residents and staff: honest, age-appropriate information; grief support arranged; staff critical incident support per the HR Policy Binder.
7	Documentation: complete factual record before end of shift; the PM manages all subsequent reporting (funder written reports, licensing, any review processes). No records relating to the person are altered or destroyed.

PART 8 — Documentation, Review and Quality Assurance

19. DOCUMENTATION STANDARDS

- Every illness or injury beyond minor first aid, every 911 call, and every event in the Section 17 matrix is documented on the Illness/Injury Report and Notification Checklist (Appendix A) before the end of the shift, in objective language: what was observed, what was done, exact times, who was notified and when.
- Injuries are documented with the Appendix E body map where location matters (all injuries of unknown origin; anything beyond trivial). Head injuries carry the Appendix B observation record until complete.
- Verbatim matters: advice from 811, Poison Control, or a prescriber is recorded in the words given; explanations for injuries are recorded as quotes with attribution.
- Records are filed in the individual service record; nothing is altered after the fact — corrections follow the single-line convention of ECC-MED §6; funder written reports use the funder's forms and are cross-referenced to Appendix A.

20. PM REVIEW AND CORRECTIVE ACTION

- The PM reviews every Appendix A report within 24 hours: completeness, response quality, and — line by line — that every Section 17 notification was made on time.
- Every emergency, hospitalization, or death is reviewed within 5 business days (jointly with ECC-HC §22 where hospital care occurred): was recognition timely; was the escalation ladder used well; did the Emergency Information Sheet work; were contacts reachable; did anything delay 911 or notification; what changes.
- Each review with findings generates at least one corrective action with a named owner and target date, tracked in the Corrective Action Log and reported quarterly to the ED — mirroring ECC-MED §31.
- Quarterly trend report to the ED: event counts and types, 911 calls, hospitalizations, notification timeliness, patterns by person/place/time, and corrective action status.

21. QA INDICATORS, ANNUAL REVIEW AND TRAINING

Indicator	Target	Frequency
First aid currency — all staff;	100%	Monthly staff currency check (by

Indicator	Target	Frequency
certified staff on site every shift		the 5th)
Emergency Information Sheet (Appendix D) current for every person	100%; updated within 1 business day of change	Monthly PC audit; verified at handover
Appendix A reports completed before end of shift; PM review within 24 hours	100%	Per event; monthly audit
Section 17 notifications made within required timelines	100% — any miss is itself an incident and is reported as such	Per event; PM personal confirmation
Head injury observation records (Appendix B) completed in full with signed handovers	100%	Per event
Emergency drill / tabletop completed	At least annually per program (and at opening)	Annual; findings actioned
First aid kits and emergency equipment checks	100% monthly checks complete	Monthly facility checklist
PM 5-business-day reviews of emergencies/hospitalizations	100%	Per event
Corrective actions closed by target date	≥ 90%; overdue actions escalated to ED	Quarterly

- This manual is reviewed annually by July 1 by the PM and approved by the ED, and immediately upon any funder or licensing directive, any death or serious incident, or regulatory change. The Section 17 matrix is reconciled to the current MCFD reportable circumstances list and the executed CLBC contract at every review.
- Orientation (before unsupervised duties) covers: the escalation ladder; the Section 14 table and emergency sequence; the head injury protocol; injuries of unknown origin and protection duties; the Section 17 matrix; and the location of kits, Appendix D sheets, and posted references. Annual refresher for all staff; competencies confirmed by the PC and recorded (Appendix G).

PART 9 — Appendices

APPENDIX A — ILLNESS / INJURY REPORT AND NOTIFICATION CHECKLIST

Complete before end of shift for every illness or injury beyond minor first aid, every 911 call, and every Section 17 event. Objective language: what was observed, what was done, exact times. Attach Appendix B (head injury) and Appendix E (body map) where applicable. PM reviews within 24 hours.

Person's Legal Name	
Date and Time of Onset / Event	
Date and Time Discovered / Reported	
Location of Event	

Type (circle): Illness / Injury / Head injury / Choking / Anaphylaxis / Seizure / Suspected overdose / Mental health crisis / Other				
Description — what was observed (signs, symptoms, mechanism of injury; quotes verbatim with attribution)				
Injury of unknown origin or inconsistent with explanation? (If yes: Section 13 applies — record protection steps below)				
First aid / immediate care provided — by whom, when, person's response				
Clinical advice sought: 811 / prescriber / Poison Control — time; advice received (verbatim)				
911 called? (time; EMS arrival; outcome) / Urgent care or ED? (facility; accompanied by; Appendix D sheet sent?)				
Outcome — condition at end of shift; admitted?; discharge instructions attached?				
Notification (per Section 17 matrix)	Required? (Y/N)	Name Notified	Date / Time	By
PC / on-call supervisor (within 30 minutes)				
Program Manager				
MCFD social worker (M) (after hours 1-800-663-9122)				
Licensing Officer (M — licensed facilities only)				
CLBC (C) — verbal same day + written CI per contract				
Consent contacts / family (per ECC-HC framework)				
CFCSA s.14 report (M) / Adult Guardianship designated agency (C) — if applicable				
Police (if applicable)				
Written funder report completed (form; date/time submitted; by)				
Follow-up actions (appointments per ECC-				

HC App. E; new prescriptions per ECC-MED §8; restrictions; observation schedules)	
Staff Member — signature / date	
PC Signature / Date	
PM Review — signature / date (within 24 hours)	

APPENDIX B — HEAD INJURY OBSERVATION RECORD

Start for any head injury beyond a trivial bump (Section 12.1). Default schedule unless clinical advice directs otherwise: hourly × 4 hours, then every 4 hours to 24 hours (waking checks overnight if directed). ANY red flag at ANY check → 911. Hand over verbally at every shift change; incoming staff sign acceptance.

Person's Legal Name						
Date / Time of Injury						
Mechanism (what happened)						
Clinical advice obtained (811 / prescriber — name, time, advice verbatim, schedule directed)						
RED FLAGS — 911 IF ANY: loss of consciousness • repeated vomiting • worsening/severe headache • confusion or unusual drowsiness • seizure • unequal pupils • weakness/unsteadiness • fluid or blood from ears/nose • person on anticoagulants						
Date / Time	Alert / rousable?	Behaviour vs baseline	Headache?	Vomiting?	Pupils (as trained)	Initials
Shift handover acceptance — incoming staff signature / date / time (each handover until complete)						
Outcome at completion (24 hours or per						

advice) — condition; any care sought	
PC Review — signature / date	

APPENDIX C — MEDICAL EMERGENCY QUICK REFERENCE (POST BESIDE EVERY PHONE)

CALL 911 IMMEDIATELY FOR	KEY ACTION
Unresponsive or hard to rouse · difficulty breathing · choking not relieved · blue lips	First aid per training; do not leave the person
Anaphylaxis — throat swelling, hives + breathing trouble	EpiPen FIRST (orange to outer thigh, hold 10 sec), then 911 — never wait
Suspected overdose or poisoning	911 + Poison Control 1-800-567-8911; Naloxone if opioids suspected and trained
Seizure over 5 min / first seizure / repeated seizures	Do not restrain; protect head; time it
Chest pain · stroke (FAST) · major bleeding · serious burns · suspected spinal injury	Keep still; pressure on bleeding; note times
Suicide attempt / serious self-harm	Stay with the person; remove dangers safely
Sepsis signs: infection + confusion, rapid breathing, mottled skin, 'worst ever'	Say 'possible sepsis' to the dispatcher
OTHER NUMBERS	
HealthLink BC — 24/7 nursing advice	811
BC Poison Control (24/7)	1-800-567-8911
On-Call Supervisor	[posted weekly schedule]
Program Prescriber / Clinic (after hours)	[insert]
MCFD Centralized Screening — after hours (M)	1-800-663-9122
CLBC contact per contract (C)	[insert]
Licensing Officer (licensed facilities)	[insert]
Nearest Emergency Department / UPCC (address + route posted)	[insert]

APPENDIX D — EMERGENCY INFORMATION SHEET (PER PERSON)

One page per person. Kept where all staff can retrieve it in seconds; a copy travels with the person in every emergency and transfer. Updated by the PC within 1 business day of any change; synchronized with the MAR (ECC-MED) and the Health Care Profile (ECC-HC App. A). Verified present at every shift handover.

Person's Legal Name / Preferred Name	
Date of Birth	

PHN	
KNOWN ALLERGIES AND REACTIONS (prominent)	
Diagnoses and health conditions (incl. seizure disorder, diabetes, cardiac, anaphylaxis risk)	
Current medications — name, dose, frequency (synchronized with MAR; date last verified)	
Emergency medications and location (EpiPen / rescue inhaler / rescue seizure medication / Naloxone / glucagon)	
Baseline and communication — how this person communicates, shows pain, is calmed; key words/supports	
Consent framework and contacts (M: social worker + team leader + after-hours 1-800-663-9122; C: committee / representative / TSDM contacts per ECC-HC App. C)	
Primary care prescriber — name, clinic, phone, after-hours	
Pharmacy	
Religious / cultural considerations relevant to emergency care (ECC-HC §29)	
Prepared by (PC) — signature / date	
Last updated — date / initials	

APPENDIX E — INJURY DOCUMENTATION BODY MAP

Use for all injuries of unknown origin and any injury beyond trivial. Mark each finding on the diagram with a number; describe each numbered finding below (size in cm, colour, shape, type — bruise/abrasion/laceration/burn). Record explanations verbatim with attribution. Attach to Appendix A.

Person's Legal Name		
Date / Time of Examination / By		
Body Diagram (front / back — mark findings 1, 2, 3...)		
[FRONT VIEW — head, trunk, arms, legs]	[BACK VIEW — head, trunk, arms, legs]	

#	Location	Description (size cm / colour / shape / type)	Explanation given (verbatim; by whom)
1			
2			
3			
4			
Photographs taken? (per program practice and consent framework — ECC-HC §29)			
Staff signature / date			
PC signature / date			

APPENDIX F — NOTIFICATION MATRIX POSTER

Post in the staff area beside the current MCFD reportable circumstances list (M) and the CLBC contract critical incident terms (C). The funder documents govern where stricter.

SERIOUS ILLNESS • SERIOUS INJURY • 911 CALL • HOSPITALIZATION	WHO / WHEN
1. PC / on-call	Immediately (within 30 minutes)
2. Program Manager	Within 30 minutes (via PC)
3. (M) MCFD social worker — after hours: 1-800-663-9122	Same day • written report within 24 hours
4. (M) Licensing Officer — LICENSED FACILITIES ONLY	Per RCR reportable incident requirements
5. (C) CLBC — verbal, then written critical incident report	Verbal same day • written per contract (commonly 24 hours)
6. Consent contacts / family (per ECC-HC framework)	Same shift
7. Protection duties where triggered: CFCSA s.14 (M) / Adult Guardianship designated agency (C) / police	Immediately — cannot be delegated or delayed
PM personally confirms every line above was completed and documented	Every event

APPENDIX G — STAFF ACKNOWLEDGEMENT

All staff must read and sign this acknowledgement before unsupervised duties and upon each annual update of this manual.

I acknowledge that I have received, read, and understood the Evergreen Community Care Illness, Injury & Medical Emergency Policy Manual (ECC-IIME-2026-001), Version 1.0, effective July 1, 2026.	Initials
I call 911 first for any emergency — consent, notification, documentation, and supervisor permission always come after — and I will never be criticized for calling 911 in good faith.	
I know the escalation ladder (observe → 811/prescriber → urgent care → 911) and that when two staff disagree about severity, the higher assessment governs.	
I act on every symptom report, monitor wellness every shift, and treat unexplained behaviour change as a health question first.	
I know the head injury red flags and the Appendix B observation procedure, including signed handover between shifts.	
I know that serious illness or injury requires notification of the MCFD social worker and licensing where the facility is licensed (M), or CLBC per the contract (C), and I start the chain by notifying the PC immediately.	
I understand my personal, non-delegable duty to report under CFCSA s.14 (M), and the adult protection reporting route (C).	
I know where the first aid kits, emergency medications, and each person's Emergency Information Sheet (Appendix D) are kept.	
I complete the Illness/Injury Report (Appendix A) before the end of my shift for any reportable event, in objective language with exact times.	
Staff Name (print)	
Position	
Program	
Signature	
Date	
Current First Aid Certificate Expiry	
PC Signature (training current, on file) / Date	

VERSION HISTORY

Version	Date	Summary
1.0	July 1, 2026	Initial standalone Illness, Injury & Medical Emergency Policy Manual (expands Part 3 of ECC-NIP-2026-001 into a full manual): prevention and preparedness

Version	Date	Summary
		(first aid readiness, per-person Emergency Information Sheets, drills, on-call system); wellness monitoring including recognition in people who do not self-report; illness escalation ladder with symptom-specific guidance; injury response with a structured head injury protocol and observation record; injuries of unknown origin with CFCSA s.14 (M) and Adult Guardianship Act (C) protection duties; 911 criteria, emergency sequence, and condition-specific protocols (anaphylaxis, choking, seizure, diabetic, overdose/Naloxone, sepsis, cardiac/stroke, mental health crisis); hospital transfer and accompaniment; Section 17 notification matrix — serious illness/injury → MCFD social worker + Licensing Officer where licensed (M) / CLBC verbal + written critical incident per contract (C) — with PM personal confirmation; death of a person in care procedures; documentation standards, PM review, corrective action, and QA indicators; 7 appendices.

Executive Director	Signature: _____ Date: _____
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